



PATIENT AND CAREGIVER EDUCATION POLICY

Stroke Unit — Comprehensive Stroke Center

Aligned with AHA/ASA Guidelines and the Get With The Guidelines®–Stroke Program

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- Approval: Stroke Medical Director / IPC / Nursing Director

This document has two parts. Part A is the governing policy for patient and caregiver education in the stroke unit. Part B provides the core content of the education materials to be adapted into patient-facing handouts, in plain language, for distribution at the bedside and at discharge.

PART A — EDUCATION POLICY

1. Purpose

This policy establishes the standard for providing structured, guideline-concordant education to stroke patients and their caregivers across the continuum of inpatient stroke care, in order to improve stroke knowledge, medication adherence, secondary prevention behaviors, safe transition to home, and early recognition of recurrent stroke symptoms.

2. Scope

This policy applies to all patients admitted to the Stroke Unit with a diagnosis of acute ischemic stroke, transient ischemic attack (TIA), or intracerebral hemorrhage, and to their identified family members or caregivers. It applies to all members of the multidisciplinary stroke team involved in patient and family education.

3. Policy Statement

Every stroke patient and, whenever possible, an identified caregiver will receive structured education covering, at minimum, the five core elements required by the AHA/ASA Get With The Guidelines–Stroke program, delivered in plain language, confirmed using the teach-back method, and documented in the medical record prior to discharge.

4. Definitions

- **Caregiver:** the family member or other individual identified by the patient as primarily responsible for providing or coordinating post-discharge support.
- **Teach-back:** a communication technique in which the patient or caregiver is asked to restate education content in their own words to confirm understanding.
- **Core education elements:** the five guideline-required discharge education topics — risk factors, activation of emergency medical services (EMS), prescribed medications, warning signs and symptoms of stroke, and need for follow-up after discharge.

5. Roles and Responsibilities

Role	Education Responsibility
Stroke Medical Director / Attending Physician	Explains diagnosis, stroke mechanism, and prognosis; approves secondary prevention plan communicated to patient/caregiver.
Stroke Program Coordinator	Oversees education program content, materials, staff competency, and compliance monitoring.
Bedside Nurse	Delivers core education elements, performs teach-back, documents education in the medical record each shift and at discharge.
Clinical Pharmacist	Provides medication reconciliation and education on new/changed medications, including anticoagulants and antiplatelets.
Rehabilitation Team (PT/OT/SLP)	Provides fall-prevention, mobility, swallowing/dysphagia, and communication-strategy education.
Dietitian	Provides diet and nutrition education, including modified-texture diets where indicated.
Social Worker / Case Manager	Coordinates discharge planning, follow-up appointments, transportation, and community resources.
EMS Liaison	Supports community-facing education on stroke recognition and EMS activation.

6. Required Core Education Content

In accordance with AHA/ASA Get With The Guidelines–Stroke performance measures, written stroke education addressing the following five topics must be provided to every patient and/or caregiver prior to discharge:

- Personal risk factors for stroke (e.g., hypertension, atrial fibrillation, diabetes, dyslipidemia, smoking, obesity).
- How to activate the emergency medical system (call emergency services) if warning signs recur.
- Prescribed medications at discharge, including purpose, dose, and adherence importance.
- Warning signs and symptoms of stroke (e.g., BE-FAST).

- The need for follow-up care after discharge, including specific appointments.

Additional Elements (Beyond the Five Core Measures)

- Fall-prevention education, provided to all patients during hospitalization (per AHA/ASA Rehabilitation Guideline).
- Caregiver-specific psychosocial education, practical training, and support resources (per AHA/ASA ICH Guideline).
- Individualized discharge planning with attention to home/environmental modification.
- Healthy lifestyle counseling: diet, physical activity, smoking cessation, and alcohol moderation.

7. Timing of Education

Phase	Education Activity
Admission (within 24 hours)	Orientation to stroke unit, diagnosis explanation, introduction of care team, initial risk-factor discussion.
Daily during admission	Ongoing reinforcement by nursing and rehabilitation staff; medication teaching as therapy is initiated or adjusted.
48–72 hours prior to anticipated discharge	Formal discharge education session with patient and caregiver; distribution of written materials; fall-prevention and home-safety review.
Day of discharge	Final teach-back of all five core elements; confirmation of follow-up appointments; provision of written after-visit summary.
Post-discharge (per follow-up protocol)	Reinforcement at outpatient neurology/stroke clinic visit and, where available, structured telephone follow-up.

8. Delivery Standards

- Education is delivered in the patient's and caregiver's preferred language, using professional interpreter services when needed — family members should not be relied upon as primary interpreters for clinical education.
- Written materials are provided at a plain-language reading level and are available in the languages most commonly used by the patient population served.
- The teach-back method is used to confirm understanding; education is not considered complete until the patient or caregiver can restate the content accurately.
- Patients with aphasia, cognitive impairment, or low health literacy receive adapted materials (e.g., pictograms, simplified text, caregiver-directed teaching) developed with Speech-Language Pathology.
- Caregivers are actively included in education sessions whenever the patient consents and a caregiver is available, consistent with AHA/ASA guidance on caregiver support after stroke.

9. Documentation Requirements

- Each education encounter is documented in the electronic medical record, including topic(s) covered, materials provided, method of delivery, and teach-back outcome.
- Completion of all five core elements is documented prior to discharge and is a required field in the discharge checklist.
- Any barriers to education (e.g., language, cognitive status, caregiver unavailability) and the plan to address them are documented.

10. Quality Monitoring

Compliance with this policy is monitored through the Stroke Program's quarterly quality dashboard, consistent with Get With The Guidelines–Stroke reporting requirements, and reviewed by the Stroke Interprofessional Committee (IPC).

Metric	Target
Stroke education (all 5 core elements) documented before discharge	≥ 95%
Caregiver included in discharge education (when available)	≥ 90%
Teach-back documented for discharge medications	≥ 90%
Follow-up appointment scheduled prior to discharge	100%

11. Special Populations

- Non-native language speakers: certified medical interpreter (in-person or video) required for all core education encounters.
- Patients with aphasia or communication impairment: Speech-Language Pathology to adapt materials and, where appropriate, lead caregiver-directed education.
- Low literacy: use of teach-back, visual aids, and simplified handouts in place of dense text.
- Patients without an identified caregiver: education focused on patient self-management, with referral to social work for community support resources.

12. References

Prabhakaran S, et al. 2026 Guideline for the Early Management of Patients With Acute Ischemic Stroke: A Guideline From the American Heart Association/American Stroke Association. *Stroke*. 2026.

American Heart Association/American Stroke Association. *Get With The Guidelines–Stroke: Discharge education performance measure specifications (risk factors, EMS activation, medications, warning signs, follow-up)*.

AHA/ASA Guideline for Adult Stroke Rehabilitation and Recovery (fall prevention, discharge planning, caregiver information).

AHA/ASA Guideline for the Management of Patients With Spontaneous Intracerebral Hemorrhage (caregiver psychosocial education and support).

PART B — PATIENT & CAREGIVER EDUCATION MATERIALS (CONTENT)

The content below is written in plain, patient-directed language and is intended as the source content for bedside handouts, discharge folders, and the patient portal. Each section corresponds to a standalone handout page.

Handout 1 — Understanding Your Stroke

A stroke happens when blood flow to part of your brain is interrupted. This can happen because a blood vessel is blocked (ischemic stroke) or because a blood vessel bleeds into or around the brain (hemorrhagic stroke). Brain cells need a constant blood supply, so a stroke can damage brain tissue quickly. This is why fast treatment matters.

- Your care team will explain which type of stroke you had and which part of the brain was affected.
- Ask your care team: What caused my stroke? What part of my brain was affected? What does that mean for my recovery?

Handout 2 — Know the Warning Signs: BE-FAST

Call your emergency number right away if you notice ANY of these signs

- B — Balance: sudden loss of balance or coordination
- E — Eyes: sudden trouble seeing in one or both eyes
- F — Face: face drooping or uneven smile
- A — Arm: arm weakness or numbness, especially on one side
- S — Speech: slurred speech or trouble speaking/understanding
- T — Time: note the time symptoms started and call emergency services immediately

Do not drive yourself or wait to see if symptoms go away. Call for an ambulance — treatment works best when given as early as possible.

Handout 3 — Your Risk Factors and How to Manage Them

Risk Factor	What You Can Do
High blood pressure	Take blood pressure medication as prescribed; monitor at home if advised; reduce salt intake.
Atrial fibrillation (irregular heartbeat)	Take anticoagulant medication exactly as prescribed; attend all follow-up appointments.
Diabetes	Monitor blood sugar as directed; follow your meal plan; take medications as prescribed.
High cholesterol	Take cholesterol-lowering (statin) medication as prescribed; follow a heart-healthy diet.
Smoking	Ask your care team about smoking cessation support and programs.
Physical inactivity	Aim for regular physical activity as approved by your care/rehabilitation team.
Obesity	Work with your dietitian on a realistic, sustainable weight-management plan.

Handout 4 — Your Medications

Before you go home, your nurse and pharmacist will review each of your medications with you. Use this table as a guide — your actual medication list will be provided separately and may differ.

Medication Type	Purpose	Key Reminder
Antiplatelet (e.g., aspirin, clopidogrel)	Helps prevent blood clots from forming	Do not stop without talking to your doctor — stopping suddenly can increase stroke risk.
Anticoagulant (blood thinner)	Prevents clots, often used for atrial fibrillation	Watch for unusual bruising or bleeding; attend all monitoring appointments.
Statin (cholesterol medication)	Lowers cholesterol and stabilizes blood vessels	Take at the same time each day; report muscle pain to your doctor.
Blood pressure medication	Controls blood pressure to reduce future stroke risk	Do not skip doses even if you feel well.
Diabetes medication	Controls blood sugar	Follow the schedule given by your care team; know the signs of low blood sugar.

Bring a complete list of your medications (or the medications themselves) to every appointment.

Handout 5 — Follow-Up Care After Discharge

- Neurology / stroke clinic follow-up appointment: scheduled before you leave the hospital.
- Primary care follow-up: recommended within 7–14 days of discharge.
- Laboratory tests: some medications (e.g., blood thinners) require regular blood tests — ask your team which tests you need and when.
- Rehabilitation therapy (physical, occupational, and/or speech therapy): attend all scheduled sessions.
- Keep a folder with your discharge instructions, medication list, and appointment dates.

Handout 6 — Preventing Falls at Home

- Remove loose rugs and clutter from walkways.
- Install grab bars in the bathroom and use a shower chair if recommended.
- Ensure good lighting, especially at night (consider night lights).
- Wear non-slip, supportive footwear — avoid walking in socks alone.
- Use any walking aids (cane, walker) exactly as instructed by your therapist.
- Ask for help or use a call button before standing if you feel unsteady, dizzy, or weak.

Handout 7 — A Guide for Caregivers

Caring for a loved one after stroke can be rewarding and demanding at the same time. This guide is for you as much as it is for the patient.

Supporting communication

- If your loved one has speech or language difficulty (aphasia), speak slowly, use short sentences, and allow extra time to respond.
- Use gestures, pictures, or writing to support communication when helpful — ask the Speech-Language Pathologist for specific strategies.

Supporting daily life

- Encourage as much independence as is safely possible — support, rather than take over, tasks your loved one can still do.
- Follow the home exercise and safety plan provided by rehabilitation therapy.

Caring for yourself

- Caregiver stress is common and expected — it is not a sign of failure.
- Ask your social worker about caregiver support groups, respite care, and community resources.
- Watch your own health, sleep, and emotional well-being; consider talking to your primary care provider if you feel overwhelmed.

Handout 8 — When to Seek Emergency Help Again

Call emergency services immediately if you notice

- Any new BE-FAST symptom, even if it goes away quickly (this may be a warning stroke/TIA)
- Sudden severe headache, unlike any before
- New confusion, severe drowsiness, or difficulty waking up
- Seizure
- Signs of significant bleeding if taking a blood thinner

When in doubt, do not wait — call for emergency help rather than driving to the hospital yourself.

Handout 9 — Healthy Lifestyle After Stroke

- Diet: follow a heart-healthy eating pattern (such as Mediterranean or DASH-style), emphasizing vegetables, fruits, whole grains, lean protein, and reduced salt.
- Physical activity: engage in regular activity as cleared by your care/rehabilitation team, building up gradually.
- Smoking: seek support to quit — ask your care team about cessation programs and medications that can help.
- Alcohol: limit alcohol intake as advised by your physician.
- Weight management: work with your dietitian on realistic, sustainable goals.
- Mental health: it is common to experience low mood after stroke — tell your care team if you notice changes in mood, motivation, or sleep.

Handout 10 — Resources & Contacts

Resource	Contact
Stroke Program Coordinator	[Insert local contact number]
Stroke/Neurology Outpatient Clinic	[Insert local contact number]
24-hour Nurse Advice Line (if available)	[Insert local contact number]
Emergency Services	[Insert local emergency number]
Caregiver Support / Social Work	[Insert local contact number]